

Maternal Morbidity Disparities Among Foreign-Born Southeast Asian Women: A Retrospective Cohort Analysis of Severe Maternal Morbidity Indicators

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Objective

To characterize the burden of severe maternal morbidity (SMM) among foreign-born Southeast Asian women delivering at a single tertiary academic center compared with US-born non-Hispanic white women and US-born Asian-American women over a ten-year period.

Study Design

Retrospective cohort analysis of 14,208 singleton deliveries at Brigham and Women's Hospital from January 2016 through December 2025. Foreign-born Southeast Asian women (n=1,847) were compared with US-born non-Hispanic white (n=8,412) and first-generation US-born Asian-American (n=3,949) cohorts using the CDC SMM indicator algorithm (21 indicators excluding blood transfusion). Multivariable logistic regression adjusted for maternal age, parity, insurance status, and comorbidity index. Subgroup analysis stratified by country of origin and years since immigration.

Results

SMM rate among foreign-born Southeast Asian women was 3.8% (70/1,847), compared with 1.9% (160/8,412) in the US-born white cohort (aOR 2.1, 95% CI 1.6–2.8) and 2.3% (91/3,949) in the US-born Asian-American cohort (aOR 1.7, 95% CI 1.2–2.4). The highest rates were observed among women within five years of immigration (5.2%, aOR 2.9, 95% CI 1.9–4.4). Hypertensive disorders of pregnancy and postpartum hemorrhage were the most common SMM indicators across all groups. Language-concordant care was associated with lower SMM in the stratified subgroup analysis (aOR 0.6, 95% CI 0.4–0.9).

Conclusion

Foreign-born Southeast Asian women delivering at our institution experienced significantly higher rates of severe maternal morbidity than both US-born white and US-born Asian-American women, with the disparity most pronounced in the first five years post-immigration. These findings support targeted prenatal outreach and language-concordant care pathways for recently immigrated Southeast Asian women. A stratified subgroup analysis by country of origin and immigration timing is ongoing.

Case example paragraph deleted per Sarah's HIPAA/IRB concern — Section 4 tightened to primary outcome composite conclusion. Stratified subgroup figure to be added if Sarah confirms.

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